



## Physiotherapy & Rehabilitation Platform

### Medical Report: Knee Rehab – ACL (Demo)

#### Patient Information

**Name:** SMEER HAKIMALI

**Birth Date:** 2006-06-08

**Case ID:** #26

#### Clinical Details

**Doctor:** Dr. Kurtoğlu

**Date:** 2026-02-03

**Description:** Post-ACL reconstruction rehabilitation, right knee

#### Physical Measurements

**Height:** 175 cm

**Weight:** 78 kg

**Blood Group:** A+

#### Health History

**Smoker:** No

**Chronic Diseases:** No

**Activity Level:** Moderate

### Additional patient information

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**Injury History:** ACL tear after football injury, surgery 6 weeks ago

**Medications:** Paracetamol as needed

**Functional Ability:** Difficulty climbing stairs and jogging

**Personal Goals:** Return to light jogging within 3 months

## Smart Clinical Analysis

### Clinical Explanation:

The patient is currently in the rehabilitation phase following an anterior cruciate ligament (ACL) reconstruction of the right knee, performed six weeks ago. The patient presents with moderate activity levels and has expressed a personal goal of returning to light jogging within three months. Functional abilities are currently limited, particularly with activities such as climbing stairs and jogging. The rehabilitation strategy will focus on restoring range of motion, strength, and functional mobility while ensuring the integrity of the surgical site.

### Immediate Physiotherapy Recommendations:

A structured rehabilitation program should be initiated, emphasizing gradual progression in range of motion exercises, strengthening of the quadriceps and hamstrings, and proprioceptive training. Incorporating low-impact activities such as cycling or swimming may also be beneficial to enhance cardiovascular fitness without placing excessive strain on the knee.

### Precautions:

The patient should avoid high-impact activities, sudden changes in direction, and any movements that cause pain or discomfort in the knee. Close monitoring of swelling and pain levels is essential, and any signs of increased instability should be addressed immediately.

### Estimated Recovery Timeline:

Given the current status and rehabilitation goals, a return to light jogging is anticipated within three months, contingent upon adherence to the rehabilitation program and absence of complications.

### Risk Assessment:

Potential risks include re-injury of the ACL, development of joint stiffness, and muscle atrophy. It is crucial to monitor for any signs of complications, such as increased swelling or pain.

### Red Flags:

Emergency warning signs include severe pain that is not relieved by rest, significant swelling, inability to bear weight on the affected leg, or any signs of infection such as fever or increased redness around the surgical site.

# Test Group: Anthropometric Measurements

## Test: Thigh Circumference (15cm above patella)

### Clinical AI Analysis

Doctor Standard/Target:  
2.0000

Clinical Interpretation:  
The thigh circumference measurements of 7, 5, and 3 cm are significantly above the target of 2 cm, indicating muscle hypertrophy or swelling in the thigh region post-surgery.

Progress Evaluation:  
While there is a reduction in thigh circumference over time, the measurements are still substantially above the target, suggesting ongoing swelling or muscle adaptation that requires attention.

Physiotherapy Focus:  
Focus on strength training for the quadriceps and hamstrings, along with modalities to reduce swelling, such as ice therapy and elevation.

HIGHEST  
7.0

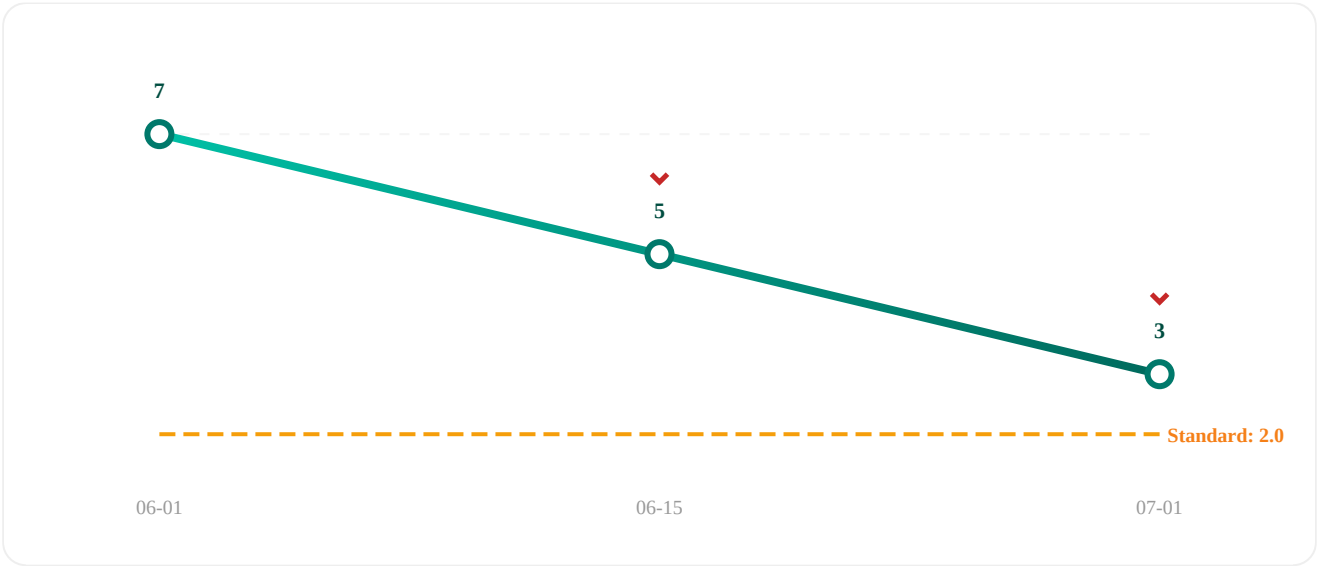
LOWEST  
3.0

AVERAGE  
5.0

STANDARD / TARGET  
2.0

### OVERALL PROGRESS

Decreased by 57.1% ↓ Since first test (2026-06-01)  
Distance to target: 1.0



| Date       | Result | Standard |
|------------|--------|----------|
| 2026-06-01 | 7      | 2        |
| 2026-06-15 | 5      | 2        |
| 2026-07-01 | 3      | 2        |

# Test Group: Lower Extremity Biometrics

## Test: Ankle Dorsiflexion ROM

### Clinical AI Analysis

Doctor Standard/Target:  
135.0000

Clinical Interpretation:  
The patient's results show ankle dorsiflexion ROM measurements of 95, 110, 122, and 128 degrees, which are significantly below the target of 135 degrees.

Progress Evaluation:  
There is a gradual improvement in ankle dorsiflexion ROM; however, the patient remains far from the target, indicating the need for focused stretching and mobility exercises.

Physiotherapy Focus:  
Incorporate specific ankle mobility exercises, such as wall stretches and calf stretches, to enhance dorsiflexion range of motion.

HIGHEST  
128.0

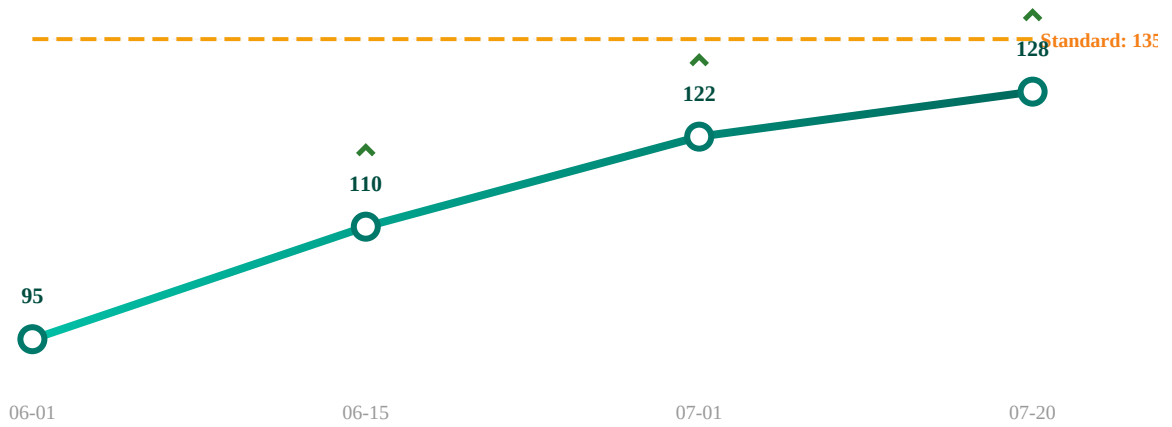
LOWEST  
95.0

AVERAGE  
113.8

STANDARD / TARGET  
135.0

### OVERALL PROGRESS

Increased by 34.7%    ↑ Since first test (2026-06-01)  
Distance to target: 7.0



| Date       | Result | Standard |
|------------|--------|----------|
| 2026-06-01 | 95     | 135      |
| 2026-06-15 | 110    | 135      |
| 2026-07-01 | 122    | 135      |
| 2026-07-20 | 128    | 135      |

### Important Notice

This report is AI-assisted and intended for informational support only. It is not a medical diagnosis and does not replace professional clinical judgment. Please consult your doctor or therapist for any medical decisions.